

DAPT for Non-Cardioembolic Ischemic Stroke

CHANCE assessed its primary outcome at 90 days; day 21 marked the end of combination aspirin–clopidogrel treatment. Trial regimens and individual eligibility are summarized below.

TRIAL	TARGET POPULATION	LOADING DOSE (DAY 1)	DAPT DURATION	POST-DAPT
POINT Trial	NIHSS ≤ 3 or ABCD ² ≥ 4 . Within 12 hours of onset.	Clopidogrel 600 mg + Aspirin 50–325 mg (162 mg/day $\times$ 5 days recommended)	Clopidogrel 75mg qD + Aspirin 50–325mg qD for 90 days (as randomized)	After the randomized comparison: individualize ongoing single-antiplatelet therapy. A fixed aspirin 81 mg regimen is not established by this trial.
CHANCE / INSPIRES	CHANCE: NIHSS ≤ 3 , ABCD ² ≥ 4 within 24h. INSPIRES: selected mild stroke/high-risk TIA with presumed atherosclerotic origin within 72 hours; qualifying imaging included culprit $\geq 50\%$ stenosis or multiple acute infarcts including unstable nonstenotic plaque. Clinical eligibility differed for ≤ 24 versus 24–72 hours; use the original criteria.	Clopidogrel 300 mg + Aspirin 75–300 mg	Clopidogrel 75mg qD + Aspirin 75–100mg for 21 days	Clopidogrel 75mg (to Day 90)
CHANCE-2 Trial	CYP2C19 LOF carrier (*2/*3) + Minor stroke/TIA. Within 24h.	Ticagrelor 180 mg + Aspirin 75–300 mg	Ticagrelor 90mg BID + Aspirin 75–100mg for 21 days	Ticagrelor 90mg BID (to Day 90)
THALES Trial	NIHSS ≤ 5 ischemic stroke, or TIA with ABCD ² ≥ 6 or ipsilateral $\geq 50\%$ stenosis, within 24 hours; apply the trial's other eligibility and bleeding exclusions.	Ticagrelor 180 mg + Aspirin 300–325 mg	Ticagrelor 90mg BID + Aspirin 75–100mg for 30 days	After the randomized comparison: individualize ongoing single-antiplatelet therapy. A fixed aspirin 81 mg regimen is not established by this trial.
SAMMPRIS Trial	Severe symptomatic atherosclerotic stenosis (70–99%) of a major intracranial artery.	Aspirin 325 mg + Clopidogrel 75 mg (no load)	Clopidogrel 75mg qD + Aspirin for 90 days	Aspirin

CYP2C19 Genotyping & Clopidogrel Resistance

- CYP2C19 LOF alleles reduce clopidogrel activation. When rapid genotype results are available, LOF status can guide ticagrelor-vs-clopidogrel selection; CHANCE-2 evidence applies to LOF carriers rather than mandating universal testing.

Safety

- **Duration — 21 vs 90 days:** Minor stroke (NIHSS ≤ 3) / high-risk TIA (ABCD² ≥ 4) → **21 days** ASA + clopidogrel (POINT/CHANCE), then single antiplatelet — benefit is concentrated in the first 21 days while bleeding exposure continues after the early ischemic benefit declines. Severe symptomatic intracranial atherosclerotic stenosis (70–99%) → **90 days** ASA + clopidogrel (SAMMPRIS) plus intensive risk-factor control. THALES ASA + ticagrelor is a **30-day** regimen for NIHSS ≤ 5 / high-risk TIA. Short-course DAPT is endorsed by the 2021 secondary-prevention guideline and the 2026 AHA/ASA AIS guideline.
- **Post-Lytic / EVT Policy:** After IV thrombolysis (tenecteplase or alteplase), avoid antithrombotics for the first 24h until follow-up imaging excludes hemorrhage. EVT alone is not a blanket DAPT contraindication; stenting/angioplasty plans and hemorrhage risk drive the decision.

POINT: Johnston SC et al. *N Engl J Med*. 2018;379:215-225. **PMID: 29766750** (<https://pubmed.ncbi.nlm.nih.gov/29766750/>) | **CHANCE:**

Wang Y et al. *N Engl J Med*. 2013;369:11-19. **PMID: 23803136** (<https://pubmed.ncbi.nlm.nih.gov/23803136/>)

CHANCE-2: Wang Y et al. *N Engl J Med*. 2021;385:2520-2530. **PMID: 34708996** (<https://pubmed.ncbi.nlm.nih.gov/34708996/>) | **INSPIRES:**

Gao Y et al. *N Engl J Med*. 2023;389:2413-2424. **PMID: 38157499** (<https://pubmed.ncbi.nlm.nih.gov/38157499/>)

THALES: Johnston SC et al. *N Engl J Med*. 2020;383:207-217. **PMID: 32668111** (<https://pubmed.ncbi.nlm.nih.gov/32668111/>) | **SAMMPRIS:**

Chimowitz MI et al. *N Engl J Med*. 2011;365:993-1003. **PMID: 21899409** (<https://pubmed.ncbi.nlm.nih.gov/21899409/>)